Muhanad Babiker

Medical History and Request for Specialist Consultation

Purpose of This Document

This document summarises the reported clinical history, MRI findings, neurological diagnoses, functional status, medication regimen, genetic investigations, and request for specialist review.
Clinical details and medication doses should be verified against the original medical records and current prescriptions.

1 Background and Clinical History

Muhanad Babiker was born in Saudi Arabia in September 2006.

At approximately 18 months of age, during a holiday visit to Sudan, he developed an illness described in the family history as viral encephalitis.

His condition was not immediately and accurately diagnosed. He was subsequently assessed at the Children's Clinic in Sudan and diagnosed with hypotonic cerebral palsy.

The clinical history associates his subsequent neurological impairment with the acute illness and prolonged untreated convulsions in El Obeid, Sudan.

2 MRI Findings

2.1 Initial MRI

The initial MRI reportedly showed diffuse cerebral atrophy and a dilated cisterna magna, without other focal changes.

2.2 Follow-up MRI: 28 February 2008

The radiology report documented the following findings:

- Moderate bilateral cerebral atrophy.
- Moderate secondary ventricular dilatation.
- Multiple periventricular abnormalities, particularly in the parietal regions.
- Atrophy of the cerebellar vermis and a degree of bilateral cerebellar atrophy.
- No dominant focal parenchymal lesion or abnormal contrast enhancement.
- The pituitary gland and brainstem were reported as normal.

Cerebral atrophy with premature periventricular leukomalacia and a degree of cerebellar atrophy. The report described features suggesting a previous hypoxic or vascular insult.

These statements represent the original radiologist's interpretation. They do not, by themselves, establish the precise mechanism or timing of the brain injury.

3 Subsequent Neurological Diagnoses

In Saudi Arabia, Muhanad was reportedly diagnosed with Lennox–Gastaut syndrome (LGS).

The reported clinical features included:

- Multiple seizure types.
- Developmental delay.
- Cognitive impairment.
- Resistance to standard treatments.

His most recent reported diagnosis by Saudi paediatric neurologists was cerebral palsy following acute necrotizing encephalopathy of childhood.

These diagnoses are reproduced from the supplied clinical history and should be verified against the original specialist records.

4 Current Functional Status

Muhanad is unable to sit independently or roll onto his side.

He can move his arms and legs, with significantly better function on the right side than on the left.

He receives F-100 therapeutic formula through a gastric feeding tube. His reported care regimen has included antiseizure medication, gastrointestinal treatment, nutritional supplements, and other medications.

5 Reported Medication and Feeding Regimen

The following medication list was supplied in the clinical history. It is not confirmation that every item remains current.

All medication names, doses, formulations, indications, and current use should be verified against Muhanad's latest prescription and medication records.

5.1 New Medications

1. Paracetamol — 400 mg three times daily.
2. Ibuprofen — 200 mg three times daily.
3. Septazole — one tablet twice daily.
4. Flagyl — 500 mg twice daily.

The strength and ingredients of Septazole were not specified in the supplied information.

5.2 Antiseizure Medications

1. Phenobarbital — 50 mg twice daily.
2. Clonazepam — 0.25 mg three times daily.
3. Levetiracetam — 900 mg twice daily.
4. Lamotrigine — 50 mg twice daily.

5.3 Gastrointestinal Medications

1. Domperidone — 10 mg twice daily.
2. Esomeprazole — 20 mg twice daily.
3. Alverine plus dimethicone (Alverinspasm) — one tablet twice daily.
4. Enterogermina probiotic — one ampoule twice daily.
5. Enemax — once every other day.

5.4 Urological Medication

1. Coli Urinal, reported as containing khellin, hexamine, and piperazine — twice daily.

The exact formulation and ingredients should be checked against the product packaging or prescription.

5.5 Feeding and Nutritional Supplements

1. F-100 formula milk — 350 mL twice daily.
2. Oral rehydration solution (ORS) — one sachet as needed.
3. Multivitamin syrup with iron (Sansovit) — 15 mL once daily.
4. Calcium supplement (Osteocare syrup) — 10 mL once daily.

The feeding volumes listed above reflect the information supplied. The complete daily feeding and hydration plan should be confirmed with Muhanad's treating clinical team.

6 Genetic Investigation

Previous records describe investigation of homozygous genomic regions on chromosomes 2 and 3, including regions containing glutamate-receptor genes.

Further genetic investigation was proposed.

The available information does not establish a confirmed genetic diagnosis or demonstrate that a particular genetic variant caused Muhanad's condition.

Review of the original genetic reports, variant classifications, and any subsequent testing results would be necessary to clarify the significance of these findings.

7 Vision

Muhanad has documented visual difficulties.

His family reports that a comprehensive assessment of his visual structures in Saudi Arabia did not identify an obvious abnormality.

Further specialist evaluation may help determine whether his visual difficulties involve cerebral visual processing, the optic pathways, the eyes, or another cause.

The available information does not establish a definitive explanation for his visual impairment.

8 Request for Specialist Consultation

I am reaching out with a deep sense of urgency and hope regarding my beloved son, Muhanad.

His history of encephalitis, prolonged seizures, severe neurological impairment, and persistent visual difficulties raises important questions about the mechanisms underlying his condition and whether additional investigations or interventions could improve his quality of life.

I would be deeply grateful for the opportunity to discuss his case and explore appropriate referrals, specialist consultations, research opportunities, or clinical trials for which he may be eligible.

In particular, I hope to obtain expert review of his MRI examinations and neurological history, clarification of his diagnoses, assessment of his visual impairment, and advice regarding further genetic or other investigations where clinically indicated.

My priority is to identify realistic, evidence-based opportunities to improve Muhanad's comfort, functional abilities, and quality of life.

I would greatly appreciate your expertise and guidance. I can provide his medical reports, MRI findings, and relevant clinical documentation for review.

Thank you for your time, consideration, and any assistance you may be able to offer.

Submitted by: Muhanad's father

Supporting documentation: Clinical reports, MRI reports and images, and relevant investigation records.

Clinical Verification Note

This document summarises information supplied by the family and descriptions from the medical records. It is not a new medical diagnosis or an independent radiological interpretation.

The medication list should be reconciled with Muhanad's treating clinician. In particular, the exact product and ingredients of Septazole and Coli Urinal, the indication for antibiotics, and the complete feeding schedule should be verified.